Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 06-Apr-2026

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1. Administrative & Study Identification

Public Study Title: Study of Inclisiran to Prevent Cardiovascular (CV) Events in Participants With Established Cardiovascular Disease **Full Official Study Title:** A Randomized, Double-blind, Placebo-controlled, Multicenter Trial, Assessing the Impact of Inclisiran on Major Adverse Cardiovascular Events in Participants With Established Cardiovascular Disease **Short Title/Acronym:** VICTORION-2 PREVENT (VICTORION-2P) **Protocol Number:** CKJX839B12302 **NCT Number:** NCT05030428 **Posting ID:** 247065760660952892 **Sponsor/ Company Name:** Novartis Pharmaceuticals **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Investigational Product:** Inclisiran (Trade Name: Leqvio | ATC Code: C10AX16) **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/leqvio-epar-product-information_en.pdf **Phase of Development:** Phase III **Medical Monitor:** Novartis Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To test the hypothesis that treatment with inclisiran sodium 300 mg s.c. taken in addition to well-tolerated high-intensity statin therapy in participants with established ASCVD will significantly reduce the risk of 3-Point-Major Adverse Cardiovascular Events (3P-MACE) defined as a composite of CV death, non-fatal myocardial infarction (MI), and non-fatal ischemic stroke. **Secondary Objectives:** To evaluate the effect of inclisiran on other cardiovascular event composites, including time to occurrence of CV death, 4P-MACE, and Major Limb Adverse Events (MALE), as well as to assess its long-term lipid-lowering efficacy (LDL-C reduction) and safety profile in this population.

2.2 Background & Rationale To determine if inclisiran, a first-in-class small interfering RNA (siRNA) targeting PCSK9, taken in addition to high-intensity statin therapy, can safely and effectively lower the risk of future cardiovascular events in patients with established cardiovascular disease. This addresses the residual cardiovascular risk in patients who fail to achieve optimal LDL-C targets despite maximizing current standard-of-care oral lipid-lowering therapies (LLT).

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target N: 17,004 participants **Number of Sites:** Multicenter (Global, ~1,000 locations) **Estimated Study Start:** 23-Nov-2021 **Estimated Primary Completion:** 13-Oct-2027

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female participants, 40 to 100 years of age. 2. Fasting LDL-C ≥ 70 mg/dL at the randomization visit. 3. Stable (≥ 4 weeks) and well-tolerated lipid-lowering regimen (with or without Ezetimibe) that must include high-intensity statin therapy (either atorvastatin ≥ 40 mg QD or rosuvastatin ≥ 20 mg QD). 4. Established CV disease defined as ANY of the following three conditions: - Spontaneous Myocardial Infarction (MI) ≥ 4 weeks from screening. - History of ischemic stroke ≥ 4 weeks prior to screening. - Symptomatic peripheral arterial disease (PAD) evidenced by either intermittent claudication with ankle brachial index (ABI) < 0.85, prior peripheral arterial revascularization procedure, or amputation due to atherosclerotic disease.

4.2 Exclusion Criteria 1. Acute coronary syndrome, stroke, peripheral arterial revascularization procedure, or amputation due to atherosclerotic disease < 4 weeks before the screening visit. 2. Treatment with PCSK9 inhibitors (e.g., evolocumab, alirocumab) within 90 days or planned use post-first study visit. 3. Planned or expected cardiac, cerebrovascular, or peripheral artery surgery or re-vascularization within 6 months after the first study visit. 4. Heart failure NYHA class III or IV. 5. Active liver disease defined as any known current infectious, neoplastic, or metabolic pathology of the liver. 6. Previous exposure to inclisiran or any other non-mAb PCSK9-targeted therapy, either as an investigational or marketed drug within 2 years. 7. Severe concomitant non-CV disease that is expected to reduce life expectancy to less than 5 years. 8. History of malignancy that required surgery, radiation therapy, and/or systemic therapy during the 3 years prior to the first study visit. 9. Pregnant or nursing (lactating) women.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Inclisiran sodium 300 mg administered on Day 1, Month 3 (Day 90), and every 6 months thereafter. **Route of Administration:** Subcutaneous (s.c.)

injection. **Duration of Treatment:** Long-term follow-up through estimated study completion (approximately 6 years).

5.2 Concomitant & Prohibited Therapy Permitted: Well-tolerated high-intensity statins (atorvastatin or rosuvastatin) with or without Ezetimibe. **Prohibited:** Other PCSK9 inhibitors during the study period.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Fasting LDL-C ≥ 70 mg/dL verification, Eligibility assessment
Baseline	Day 1	Randomization, First Subcutaneous Injection, Vitals, Baseline Labs
Interim	Month 3 (Day 90), then every 6 months	Subsequent Injections, Safety Labs, Efficacy/MACE Assessment, CV Event monitoring
End of Study	Follow-up / Year 6	Final Endpoint Assessment, Safety Follow-Up, IP Accountability

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint 1. **Time to First Occurrence of 3P-MACE (3-Point Major Adverse Cardiovascular Events):** A confirmed composite endpoint which includes cardiovascular death, non-fatal myocardial infarction, and non-fatal ischemic stroke.

7.2 Secondary & Exploratory Endpoints 1. **Time to Occurrence of Cardiovascular (CV) Death** 2. **Time to First Occurrence of 4P-MACE (4-Point Major Adverse Cardiovascular Events)** 3. **Time to first occurrence of Major Limb Adverse Events (MALE)** *(Other measures include percent change in LDL-C from baseline and extended composites of cardiovascular outcomes like peripheral arterial revascularization or urgent heart failure visits).*

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection evaluating safety and tolerability (e.g., injection site reactions, anti-drug antibody formation). **Serious Adverse Events (SAEs):** Continuously monitored and adjudicated to separate baseline CV risk from investigational product-related SAEs. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) implemented to review unblinded safety and efficacy data periodically.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy (MACE) endpoints. Safety Set for all treatment-emergent adverse events (TEAEs). **Sample Size Justification:** $N=17,004$ designed to yield adequate statistical power to detect a clinically meaningful relative risk reduction in the 3P-MACE primary endpoint between inclisiran and placebo. **Handling of Missing Data:** Time-to-event analyses evaluated using Cox proportional hazards models and Kaplan-Meier estimates, utilizing appropriate censoring for patients lost to follow-up.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote source data verification (SDV) to ensure protocol adherence across global sites. **Audit Trail:** Robust electronic data capture (EDC) systems utilized with full audit trails, strict eCRF locking mechanisms, and independent endpoint adjudication.

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11. Study Identifiers & Governance

Primary ID: CKJX839B12302 **Registry Number:** NCT05030428 **Posting ID**: 247065760660952892 **Sponsor/Lead Organization**: Novartis **Public Study Title:** Study of Inclisiran to Prevent Cardiovascular (CV) Events in Participants With Established Cardiovascular Disease **Official Regulatory Title:** A Randomized, Double-blind, Placebo-controlled, Multicenter Trial, Assessing the Impact of Inclisiran on Major Adverse Cardiovascular Events in Participants With Established Cardiovascular Disease (VICTORION-2 PREVENT)

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12. Clinical Framework (Cardiovascular Specific)

Primary Condition / Disease Area: Atherosclerotic Cardiovascular Disease (ASCVD) **Preferred Name:** Atherosclerosis **Semantic Type:** Disease or Syndrome **Definition:** A thickening and loss of elasticity of the walls of ARTERIES that occurs with formation of ATHEROSCLEROTIC PLAQUES within the ARTERIAL INTIMA. **Medical Classifications/Codes**: - **MeSH Code:** D050197 - **HPO Code:** HP: 0002621 - **SNOMED CT ID**: 38716007 - **SNOMED Hierarchy:** Arteriosclerosis > Vascular sclerosis > Degenerative abnormality > Morphologically abnormal structure > Morphologically altered structure > Body structure **Diagnosis Threshold:** Fasting LDL-C ≥ 70 mg/dL despite high-intensity statin **Medical Methodology:** PCSK9-targeted siRNA lipid-lowering therapy **Optimization Target:** Significant reduction in LDL-C to prevent MACE

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13. Study Procedures & Methodology

Management Pathway: Addition of twice-yearly inclisiran injections onto maximally tolerated oral LLT. **Education Protocol (HCP):** Administration guidelines for subcutaneous injection, reporting of CV events. **Education Protocol (Patient):** Compliance with background statin therapies, maintaining diet, and adherence to bi-annual clinical visits. **Quality Audit Mechanism:** Continuous safety data reviews by independent committees and endpoint adjudication.

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14. Observation & Follow-Up Schedule

Total Duration: Estimated ~72 months (until target number of MACE events is reached) **Onsite Visit Cadence:** Day 1, Month 3, and then every 6 months **Remote Monitoring Frequency:** As required per site protocol between bi-annual visits. **Checklist Review Interval:** Continuous event tracking for 3P-MACE components.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]